INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pembrolizumab + paclitaxel + carboplatin AUC6

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00579
Version 5a

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: paclitaxel and carboplatin stop; pembrolizumab alone may continue every 21 days while appropriate. The team agrees duration and review points.

Treatment stages

01 **Induction · up to four cycles**
Three medicines

02 **Maintenance · after induction**
One medicine

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Pembrolizumab + paclitaxel + carboplatin AUC6 · National Cancer Control Programme (NCCP) 00579

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

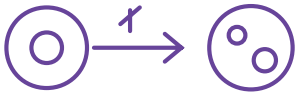
Three medicines. Different jobs.

This combination aims to control metastatic squamous lung cancer, help symptoms and extend life. Your team weighs likely benefit, risks and what matters to you.

IMMUNOTHERAPY

Pembrolizumab

Keytruda



Releases an immune brake

PD-1 is a control switch on T cells, a type of immune cell. When it connects with PD-L1 or PD-L2, the T cell receives a signal to reduce its activity. Some cancers use this pathway to escape immune attack. Pembrolizumab is an antibody that blocks PD-1, helping T cells recognise and attack the cancer. It does not directly poison cancer cells like chemotherapy. Releasing this brake can also allow immune cells to attack healthy organs, which explains the different side-effect pattern and why symptoms can appear after treatment stops.

Immune cell → Brake blocked → Cancer cell

What to watch for

The immune system can also inflame healthy organs. New cough, diarrhoea, unusual weakness, severe headache, thirst or a spreading rash need prompt advice. Effects can start during treatment or after it finishes.

CHEMOTHERAPY

Paclitaxel

Taxane chemotherapy



Disrupts cell division

Paclitaxel stabilises tiny structures called microtubules that cells need to divide. This disrupts cancer-cell division. Healthy tissues can be affected too, including blood-forming cells and nerves. That explains why infection risk, hair loss and numbness or tingling matter during treatment.

Dividing cell → Microtubules held → Division disrupted

What to watch for

Report numbness, tingling or difficulty using your hands before the next dose. During the infusion, tell the nurse immediately about itching, flushing, chest tightness, back pain or feeling faint.

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Carboplatin is a platinum-containing chemotherapy. Inside cells, it forms chemical attachments to DNA, including links that stop DNA strands working normally. This disrupts copying and repair and can lead to cell death. Healthy cells can also be injured, especially blood-forming cells in the bone marrow. The kidneys help remove carboplatin, so kidney function is used to calculate the dose. AUC 6 describes a target drug exposure over time; it is not a fixed dose of 6 mg.

DNA strands → Platinum links → Copying disrupted

What to watch for

Low blood counts, nausea and vomiting can occur. An allergic reaction is also possible: tell the nurse immediately if you feel flushed, itchy, tight-chested or unwell during the drip.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the treatment aim

Take one section at a time and discuss what matters to you.

What do the words mean?

Squamous describes the lung-cancer cells seen by the pathologist. Metastatic means the cancer has spread beyond its original site. First-line means the first drug treatment for this stage. This regimen combines chemotherapy with an immune treatment.

What is treatment trying to achieve?

The usual aims are cancer control, symptom relief and longer life while considering quality of life. It is generally not given with an expectation of cure. Some people have a prolonged response; others do not. Your team uses your cancer tests, general health and preferences to choose a plan.

What are the alternatives?

Ask whether another chemotherapy/immunotherapy option, a clinical trial or symptom-focused care is appropriate. Supportive and palliative care can accompany active treatment. PD-L1 helps inform choices, but this combination's trial included people across PD-L1 groups and a result does not guarantee benefit.

Why this treatment is offered

Adding pembrolizumab to chemotherapy improved survival and delayed progression in the trial population. Results cannot predict one person's benefit.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Pembrolizumab + paclitaxel + carboplatin AUC6 · NCCP 00579

Induction → review → maintenance

Treatment stages

01 **Induction · up to four cycles**
Three medicines

02 **Maintenance · after induction**
One medicine

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE MEDICINES

Induction · up to four cycles

C1 C2 C3 C4

Induction: all three medicines on Day 1 every 21 days for up to four cycles.

ONE MEDICINE

Maintenance · after induction

M1 M2 M3 …

Maintenance: paclitaxel and carboplatin stop; pembrolizumab alone may continue every 21 days while appropriate. The team agrees duration and review points.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. No patient dates are needed. Agree who arranges and reviews each check.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00579 v5a · official regimen

THE WHOLE TREATMENT JOURNEY

Pembrolizumab + paclitaxel + carboplatin AUC6: treatment and checks

First-line metastatic squamous non-small-cell lung cancer (NSCLC)

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: paclitaxel and carboplatin stop; pembrolizumab alone may continue every 21 days while appropriate. The team agrees duration and review points.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First induction dose	During induction	Maintenance review	After treatment
Treatment NCCP 00579	Confirm indication and planned phases	Three medicines on Day 1	Every 21 days; up to four induction cycles	Continue pembrolizumab every 21 days while appropriate	Agree follow-up and next decisions
Bloods and hormones NCCP 00579	full blood count (FBC), kidney/liver, glucose, thyroid and hepatitis B/C screening	Review results and baseline symptoms	FBC, kidney/liver and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks; review symptoms	Follow unresolved abnormalities and the team's plan
Nerves, hearing and kidneys NCCP 00579	Document baseline nerves/hearing; renal assessment	Check carboplatin renal dosing and taxane premedication	Nerve/hearing review before platinum; report new symptoms	Platinum stops; continue follow-up of any ongoing toxicity	Persistent symptoms need review
Support medicines NCCP 00579	Confirm taxane premedication and carboplatin antiemetics	Use the induction support prescription	Check adherence, effectiveness and side effects	Stop routine taxane premedication and induction antiemetics; symptom-directed plan	Follow the revised medicine plan
Immune effects and cancer review NCCP 00579	Explain alert card, symptoms and treatment aims	Report infusion symptoms immediately	Report new cough, diarrhoea, rash or endocrine symptoms between visits	Continue pembrolizumab while appropriate; scans per consultant plan	Delayed immune effects remain possible

What each check is for

1 **Before treatment**

Before first dose

Establish suitability and baseline

Ask / **act:** Confirm results, support prescriptions and responsible teams.

2 **Phase change**

After up to four induction cycles

Active medicines and support change

Ask / **act:** Confirm maintenance medicines and revised support; continue immune monitoring.

3 **After final treatment**

Completion or treatment change

Delayed effects and continuing support

Ask / **act:** Give the follow-up and urgent-contact plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments and time in the unit

Pembrolizumab first over 30 minutes, then paclitaxel over 3 hours, then carboplatin over 30 minutes during induction. Premedication and observation add time. Clinic time includes assessment, preparation and waiting; ask your unit about the full visit.

STEP 2

Treatment phase

STEP 3

Medicines and practical help

Check the paclitaxel premedication and anti-sickness plan for induction. Maintenance has different medicines and may be a shorter visit. Plan transport if medicines make you drowsy. Keep the immunotherapy alert card and urgent numbers with you.

STEP 4

After treatment

Response imaging follows the consultant plan; no fixed universal scan interval is specified in these NCCP regimens. Immune toxicity can arise after the final pembrolizumab dose. Continue symptom vigilance, immunotherapy alert card and the team's follow-up plan. Review persistent neuropathy, hearing effects and cytopenias even after chemotherapy stops; immunotherapy maintenance has a different supportive-care plan.

Before the first infusion

Your hospital's instructions take priority.

Tell the team your starting point

Describe usual breathing, bowel pattern, energy, skin and nerve symptoms. Mention autoimmune illness, previous lung inflammation, infection and all medicines including steroids and supplements. Changes are easier to recognise when the team knows what is normal for you.

Prepare the practical details

Keep the oncology and out-of-hours numbers, a thermometer and the immunotherapy alert card available. Check blood-test appointments, transport and who can help at home. Ask for written medicine directions and accessible information.

What happens on the day?

Blood results and symptoms are reviewed before release of treatment. Pembrolizumab is followed by paclitaxel and then carboplatin during induction. Premedication helps reduce paclitaxel reactions. Tell the nurse immediately about drip-site pain, itching, flushing, chest/back pain, dizziness or breathing changes.

What tests and scans check

A normal earlier test does not rule out a new problem.

Blood counts

Neutrophils help fight infection, platelets help stop bleeding, and red cells carry oxygen. Counts can fall between visits. You cannot know a count from the day of the cycle or from feeling well.

Kidneys, liver and hormones

Kidney function helps determine the carboplatin dose. Liver tests influence paclitaxel safety. Pembrolizumab can affect hormone-producing glands, so glucose and thyroid monitoring continue in maintenance. Report symptoms even if routine tests are normal.

Scans and treatment review

The team sets the scan schedule and reviews scans with symptoms and examination. Stable cancer can be a worthwhile result. Report new symptoms before the next scan; do not assume worsening is evidence that immune treatment is working.

Delays and changed doses

Chemotherapy may be delayed or reduced for toxicity. Pembrolizumab is held or stopped for relevant toxicity rather than dose-reduced. Ask which medicine is changing and why. Omitted checks are not the same as normal results.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Pembrolizumab + paclitaxel + carboplatin AUC6 · NCCP 00579

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment	 **Day 1** Day 1 · infusion visit
Nausea / appetite	 **Days 1–5** Early days · may continue longer
Blood counts / infection	 **Days 1–21** Counts may fall between visits · timing varies
Energy / mouth / bowels	 **Days 1–21** Symptoms vary across the cycle
Immune effects	 **Days 1–21** Any day · also after treatment ends

Induction and maintenance differ: Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: paclitaxel and carboplatin stop; pembrolizumab alone may continue every 21 days while appropriate. The team agrees duration and review points.

After the final infusion: Response imaging follows the consultant plan; no fixed universal scan interval is specified in these NCCP regimens. Immune toxicity can arise after the final pembrolizumab dose. Continue symptom vigilance, immunotherapy alert card and the team's follow-up plan. Review persistent neuropathy, hearing effects and cytopenias even after chemotherapy stops; immunotherapy maintenance has a different supportive-care plan.

Symptoms do not follow a fixed calendar: Effects may overlap between cycles. Immune symptoms can occur after treatment stops. The diagram cannot identify a safe day or predict recovery.

> **Why these bands stay broad:** The treatment bands show a reference 21-day cycle. Induction and maintenance contain different medicines. Symptom bands are broad educational illustrations, not measured daily risks or a prediction of when you will feel well; immune-related effects can occur after treatment ends. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

Between-treatment notes

Day 1: Your infusion day

Your team checks symptoms and blood results before treatment. Tell your nurse immediately about pain at the drip site, itching, flushing or breathing changes. The time in hospital includes checks and preparation, not just infusion time. Before leaving, check your take-home medicines, contact details and next appointment.

Day 2–5: The first days at home

Feeling sick can start on treatment day or later. Appetite, taste, sleep or bowel habits may change. Take prescribed supportive medicines as directed; ask for help if they are not controlling symptoms. Contact the team if vomiting persists, you cannot keep fluids down, or you develop new diarrhoea. Do not assume diarrhoea is ordinary chemotherapy upset.

Day 6–14: Keep checking how you feel

Blood counts may be reduced between treatments. You cannot tell your count from the date or from how well you feel. Fatigue or mouth symptoms may affect eating and daily activity. Use the urgent advice below for fever, shivering or suddenly feeling unwell, including with a normal temperature.

Day 15–21: Prepare for your next review

Some symptoms may settle; others can continue or appear for the first time. Improvement does not prove your blood counts have recovered. The next treatment depends on your review and tests. Bring your symptom record and questions. Confirm the next appointment: the next Day 1 is usually 21 days after this cycle's Day 1, if treatment proceeds.

Support around the medicines

1 Induction support

Anti-sickness medicines help with carboplatin. Paclitaxel premedication helps prevent infusion reactions. Ask for one clear steroid schedule and report infusion symptoms immediately.

2 Maintenance support

Chemotherapy and taxane premedication stop. Pembrolizumab alone usually does not need the same preventive anti-sickness medicines; ask for the updated prescription. Immune symptom vigilance continues.

Side effects and what to do

Selected possibilities are shown; this is not a prediction or a complete medicine leaflet.

Nausea, vomiting and appetite

Take prescribed anti-sickness medicines as directed. Small meals and regular sips may help. Report poor control early; persistent vomiting, inability to keep fluids down or much less urine needs prompt advice.

Fatigue and weakness

Balance rest with manageable activity. Sudden severe weakness, dizziness, breathlessness or inability to manage basic tasks needs assessment: infection, anaemia or hormone problems can resemble ordinary tiredness.

Mouth and bowels

Use gentle mouth care and report painful ulcers or difficulty drinking. Report new diarrhoea, abdominal pain or blood in stools promptly because immune bowel inflammation is possible. Ask about constipation from anti-sickness drugs or pain medicines; severe pain, swelling or vomiting needs assessment.

Hair, skin, muscles and nerves

Paclitaxel commonly causes hair loss and may cause muscle/joint aches and numbness or tingling. Report nerve symptoms before the next dose, especially if balance or hand function changes. Blisters, peeling skin or sores around eyes/mouth need urgent advice.

Bruising and bleeding

Report unusual bruising, nosebleeds or bleeding gums. Heavy bleeding, black stools with weakness or severe breathlessness needs urgent assessment. Do not change prescribed blood thinners without advice.

Pembrolizumab: symptoms that deserve special attention

Immune treatment can cause inflammation in healthy organs, during treatment or after it ends. Keep your immunotherapy alert card and tell other healthcare professionals you have received pembrolizumab.

Lungs, bowel, liver and kidneys

New cough or breathlessness, diarrhoea, yellow skin or eyes, dark urine, reduced urine or swelling can signal inflammation. These symptoms also have other causes, including infection and cancer. Contact the team promptly for assessment rather than trying to identify the organ or diagnosis yourself.

Hormones, heart and nervous system

Report unusual weakness, persistent headache, dizziness, marked thirst, frequent urination, palpitations, muscle weakness or changes in vision. Severe chest pain, collapse, confusion, seizures or major breathing difficulty require emergency care. Some hormone effects need ongoing replacement treatment even after immunotherapy stops.

Why early reporting matters

The team may pause treatment and investigate. Some reactions need steroids or other immune-suppressing treatment; others need hormone replacement. These are different from the short planned steroid course around chemotherapy. Do not start leftover steroids, change their dose or stop a prescribed course yourself. Carry the treatment name when attending emergency care.

Living with treatment

The aim is a plan that fits your life as well as your treatment. Bring practical concerns to the team early.



Food, fluids and weight

Aim for enough food and fluid to maintain strength, using foods you tolerate. Tell the team about weight loss, swallowing problems or difficulty shopping or cooking; ask for dietitian support. A fixed fluid target is not right for everyone, particularly with heart or kidney disease. “Anti-cancer” diets, fasting and high-dose supplements are not substitutes for treatment and can create nutritional or interaction problems.

Movement, work and rest

Try activity that feels manageable and increase or reduce it according to symptoms. Short walks or simple daily tasks may be more realistic than your usual exercise. Discuss work adjustments and travel plans. Do not drive if dizzy, unusually tired or affected by sedating medicines. Sudden deterioration should prompt assessment, not an attempt to push through exercise.

Infection precautions without isolation

Wash hands, practise food hygiene and avoid close contact with people who are unwell. Ask about vaccinations and their timing; some vaccines are unsuitable during treatment. You can usually still spend time with family. Keep your urgent contact plan available: precautions reduce risk but do not make fever or sudden illness safe to watch at home.

Intimacy, fertility and pregnancy

Discuss fertility preservation before treatment if relevant. Treatment may harm a developing baby; ask for a written contraception plan covering all three medicines and the period after the last dose. Do not breastfeed during treatment; confirm when it would be safe to restart. Tell the team promptly about a possible pregnancy. Ask about barrier protection and handling body fluids under your hospital's advice.

Emotional support and choices

It is reasonable to feel worried, angry, uncertain or overloaded. Ask for psycho-oncology, counselling, social work or family support. You can request information in smaller pieces, an interpreter or a written summary. Tell the team what you most want to preserve in daily life. You can revisit treatment decisions and ask for more time or another explanation.

A short treatment dictionary

Terms you may hear in clinic.

Cycle · induction · maintenance

A cycle is a planned treatment-and-interval period. Induction is the initial combination phase. Maintenance is ongoing treatment after that phase; it does not mean side-effect-free treatment.

Response · stable disease · progression

Response means the cancer has reduced by the assessment used. Stable disease means it has not changed enough to meet response or progression criteria. Progression means growth or spread meeting the assessment criteria. Your team interprets these alongside how you are feeling.

Neutropenia · anaemia · thrombocytopenia

Neutropenia means low infection-fighting neutrophils. Anaemia means low haemoglobin. Thrombocytopenia means low platelets. These are different results with different implications; one normal number does not establish that all treatment risks are low.

Immune-related · colitis · pneumonitis

Immune-related effects come from immune activity against healthy tissue. Colitis means inflammation of the colon; pneumonitis means inflammation in the lungs. A symptom alone cannot establish either diagnosis.

Questions worth asking

How will we know if it is helping?

Ask your oncologist what the treatment is aiming to achieve for you, when scans are planned and what would lead to continuing, changing or stopping it. How you feel and the amount of side effects do not reliably show how well treatment is working.

Why might a cycle be delayed?

A delay can give time to recover or investigate a new symptom. It is a safety decision, not a personal failure. Ask which medicines are affected, what needs to improve and when you will be reviewed.

Will I get every side effect?

No. The body map shows selected possibilities, not a prediction. Ask about common effects, serious less common effects, and any long-term risks that matter particularly to you. The same symptom can have several causes.

What should I organise before starting?

Confirm blood-test and Day Ward arrangements, transport home, your medicines schedule and a thermometer. Keep the oncology contact instructions and immunotherapy alert card available. Ask for an interpreter or accessible information if useful.

What else belongs in the consent conversation?

Your priorities, expected benefit, alternatives including supportive care, fertility, contraception, pregnancy or breastfeeding, work, driving and the possibility of lasting side effects. You can ask more questions or reconsider treatment at any stage.

Do more side effects mean the treatment is working better?

No. Side effects are not a reliable personal measure of cancer response. Report them so they can be treated; do not tolerate serious symptoms in the hope that they are a sign of benefit.

What should I say when calling about a symptom?

Give the current medicines or say you are in maintenance, the last treatment date, what changed and when, your temperature and whether you can eat, drink and pass urine. Do not delay calling to collect every detail.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Pembrolizumab

- Immune inflammation of lungs, bowel, liver, kidneys, glands, skin and other organs
- Serious infusion reactions
- Effects can develop after treatment ends

Paclitaxel

- Low blood counts and infection
- Hair loss
- Numbness or tingling that may persist

Carboplatin

- Low blood counts, infection or bleeding
- Allergic reactions
- Kidney, hearing and nerve effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: paclitaxel/carboplatin cycles 1–4

Active medicines: pembrolizumab + paclitaxel + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk and patient factors.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	NCCP NCIS V8 high-risk dedicated carboplatin AUC≥4 row Reconcile with paclitaxel premedication steroid and aprepitant interaction; issue one clear prescription, do not add overlapping courses.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS table does not specify an end day here; prescribe duration and suitability. Not a routine olanzapine-containing row.
Paclitaxel hypersensitivity premedication	Source suggested dexamethasone 20 mg orally or intravenous (IV); chlorphenamine 10 mg IV; famotidine 20 mg IV	Oral dexamethasone at 6 and 12 hours before paclitaxel, or IV 30 minutes before; H1/H2 30 minutes before With aprepitant source permits considering dexamethasone 12 mg on treatment day. Reconcile antiemetic and taxane steroids; do not stack doses. Later reduction/omission or H2 omission requires a consultant/local plan and monitoring, particularly with this 3-weekly schedule.

Additional phase-specific support

- Use the dedicated carboplatin AUC≥4 high-risk NCIS row; it has no routine olanzapine.
- Paclitaxel premedication prevents infusion reactions and must be reconciled with antiemetic dexamethasone.
- Manage muscle/joint aches according to a prescribed analgesic plan; assess neuropathy before each taxane cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Maintenance: pembrolizumab

Active medicines: pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Pembrolizumab alone has minimal emetic risk: no routine continuation of the induction antiemetic course or taxane premedication.
- Prescribe symptom-directed rescue if needed and investigate unexplained nausea/vomiting for immune or other causes.
- Continue immune, renal/liver, glucose and thyroid monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00579 · VERSION 5A

Pembrolizumab + paclitaxel + carboplatin AUC6

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: paclitaxel and carboplatin stop; pembrolizumab alone may continue every 21 days while appropriate. The team agrees duration and review points.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hormones / brain

Pembrolizumab

Unusual weakness, dizziness, headache, thirst or frequent urination: report promptly.

2 Lungs

Pembrolizumab

New cough or breathlessness needs prompt assessment; infection, cancer or clot can look similar.

3 Heart

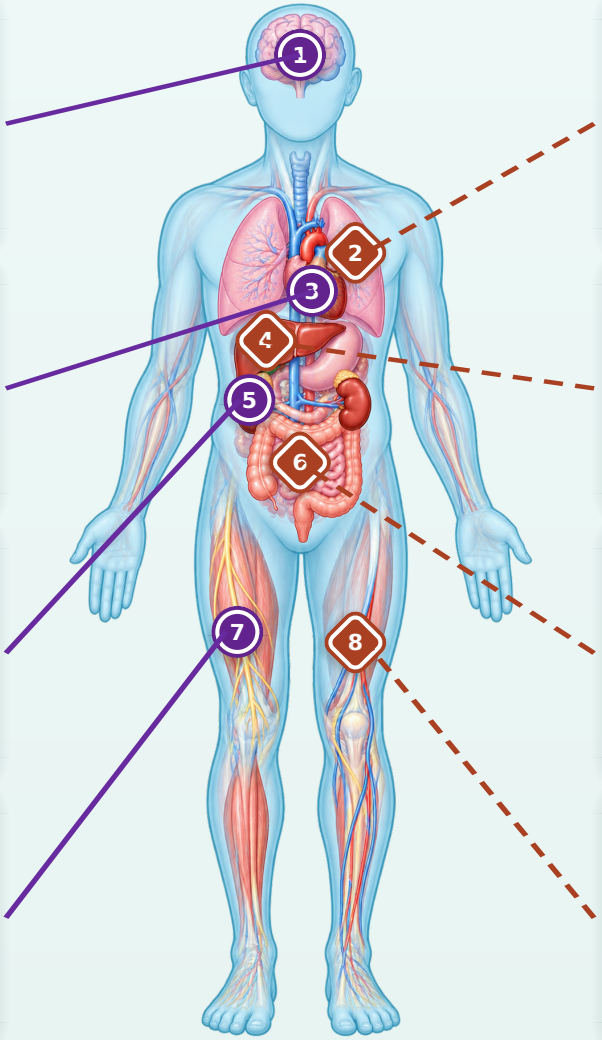
Pembrolizumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

4 Liver

Pembrolizumab / paclitaxel

Yellow skin/eyes or dark urine; tests also check changes before symptoms.



5 Kidneys

Pembrolizumab / carboplatin

Report much less urine, swelling or persistent vomiting; kidney results affect treatment.

6 Bowel / mouth

Pembrolizumab / chemotherapy

New diarrhoea, severe tummy pain, mouth ulcers or inability to drink need advice.

7 Skin / nerves

Pembrolizumab / paclitaxel

Report rash, blisters, numbness, weakness or problems with balance or hand function.

8 Blood / infection

paclitaxel / carboplatin

Fever, shivers or suddenly feeling unwell: contact oncology immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

SACTCHECK · CHAPTER 04

Sources and further information

Pembrolizumab + paclitaxel + carboplatin AUC6 · National Cancer Control Programme (NCCP) 00579

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00579 v5a · pNCCP 00579 treatment and supportive-care sections; NCIS V8**
   https://healthservice.hse.ie/documents/6887/579_V5a_pembrolizumab_carboplatin_paclitaxel.pdf
2. **NCCP NCIS Medical Oncology antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **KEYNOTE-407 five-year published follow-up**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10082300/
4. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/